Health Connect



Team 1

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Hospital discharge is never easy.

Logistical hurdles are everywhere.

Inefficient Process: Scheduling transportation + waiting for paperwork

Highly manual: Nurses + case managers coordinating everything

Error-prone: Missed meds, delayed equipment, readmissions → $$$



Hospital discharge is never easy.

Logistical hurdles are everywhere.

Wastes time + money
when time + money
are *critical*.

Current Tools

Patient Portals	Care Coordination Platforms	Comm. + Collab. Tools
These are patient-facing record viewers, **not operational systems**.	These are coordination dashboards, but **not deeply automated**.	These solve communication, **not decision-making** or execution.
- Show patient records, meds, appointments - Messaging with providers - Billing + scheduling	- Track patients across settings - Manage referrals + discharge planning - Assign tasks to care teams	- Messaging, alerts, coordination across staff - Real-time updates from EHR systems
Ex: MyChart, Salesforce Health Cloud, FollowMyHealth, Phreesia	Ex. WellSky, Cerner HealtheIntent, Persivia CareSpace, Netsmart	Ex. TigerConnect

Hospital discharge is never easy.

Existing tools *show* information and are *patient-focused*,

but don't ***execute*** discharge.

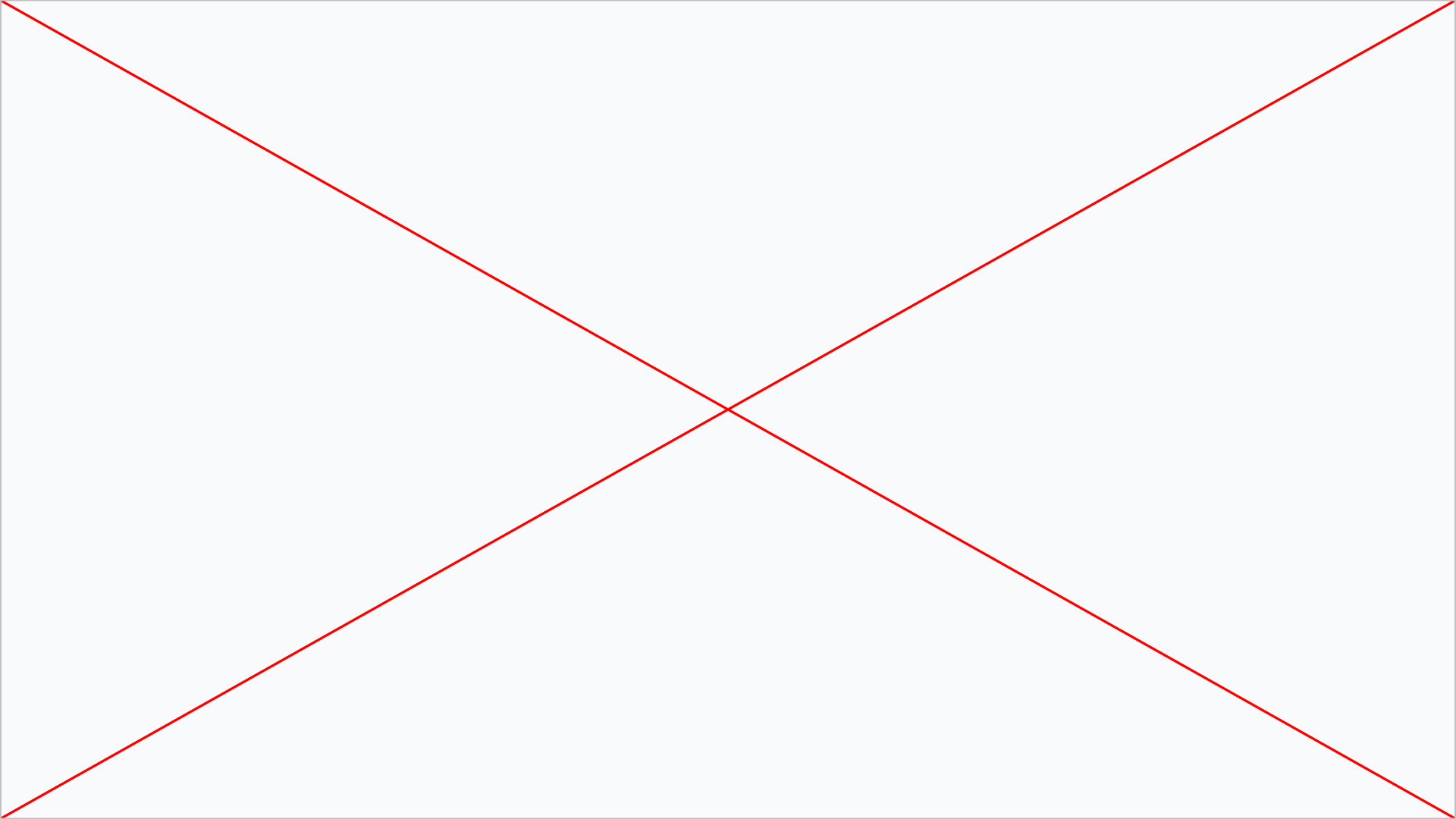
👋 Meet...

➕ Health Connect

Orders your prescriptions.

Books your appointments.

Dispatches equipment to home.



Fairness, Accountability, Transparency, Ethics

Humans have the final say.

Agent is logged and traceable

Requires human sign-off

We build for trust.

Consistent protocols applied across every patient

HIPPA → Encryption

RISKS	MITIGATION
HIPAA & Unauthorized Access — sensitive Protected Health Information (PHI) collected at scale	Business Associate Agreements (BAAs) with all vendors, encrypted pipelines, role-based access
AI Inference Errors — wrong medication or treatment compiled	Human review required before any clinical action is confirmed
Electronic Health Record (EHR) Integration — Epic approval is slow and hospital-by-hospital	Start with read-only Fast Healthcare Interoperability Resources (FHIR) APIs, write-back in later phases
Liability — who's accountable if something is missed?	Agents assist and log but final sign-off stays with clinical staff
Sales Cycles — hospital procurement takes 12–18 months	Target Hospital Readmissions Reduction Program (HRRP)-penalized hospitals with a direct ROI story

Technical Feasibility

Overall technical feasibility: High ✅

Epic FHIR Data

- Structured clinical inputs (meds, allergies, diagnoses)

Preprocessing

- Standardize meds, map diagnoses, extract relevant chart sections

Agent Engine

- Each agent handles one discharge step with human sign-off

LLM Layer

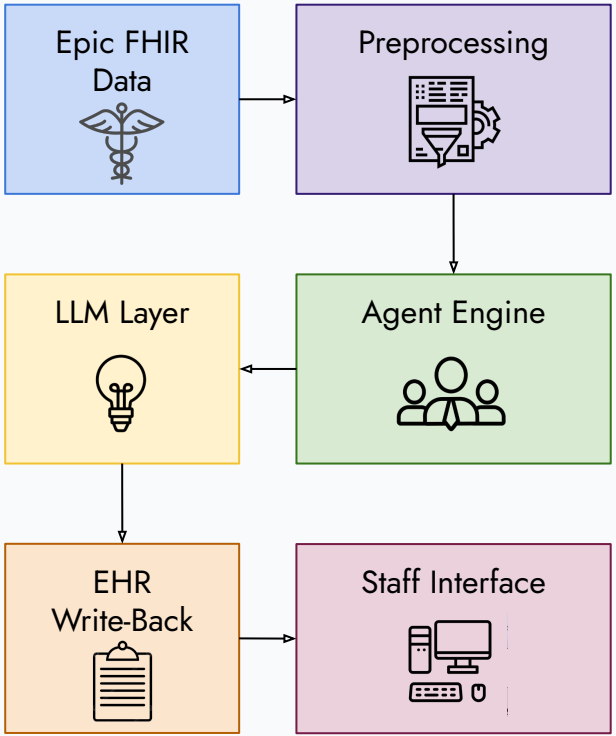
- Conflict explanations, summaries, communication drafting

EHR Write-Back

- Appointments, provider messages, discharge documentation

Staff Interface

- Clear, actionable steps for nurses, pharmacists, and physicians



Build Timeline and Costs

Total Est: $744K – $1.4M

Build Costs

2 engineers x 12 months

$350,000

2,000-2,500 hrs for HIPAA-compliant MLP for $150K salary

EHR Integration

Epic FHIR APIs

$350,000 - $1,000,000

Depends on usage of Epic's free FHIR APIs or custom integration

HIPAA Integration & Maintenance

One-time + ongoing

$40,000 - $60,000

Covers: Privacy, Security, Enforcement, Breach Notification
Maintenance costs of **$4,000 to $6,000**

Token & API Cost Estimate (per discharge)

Agent Task	Input Tokens	Output Tokens	Est. API $
Patient Summary (meds, etc.)	~3,000	~500	$0.02
Medication Reconciliation	~4,000	~800	$0.03
Auto-Scheduling	~2,000	~400	$0.02
Patient Comms (SMS, etc.)	~1,500	~600	$0.01
Total per discharge	~10,500	~2,300	$0.08

37M U.S. discharges/year → API cost of <$3M/yr

Build Phases

1 Months 1 - 6

FRIR read-only
Patient Summary
Med Reconciliation

2 Months 7 - 12

Auto-scheduling
Patient comms
SMS/email/call

3 Year 2+

EHR write-back
Instruments dispatch
Full Epic app cert.

Financial Feasibility

TAM

Total Addressable Market

~$1.85B

37M discharges/yr
x $50/discharge

Serviceable Addressable Market (SAM)

~$350 million

HRRP-penalized medium-to-large hospitals → CMS fines hospitals with high readmission rates)

Serviceable Obtainable Market (SOM) Year 1

~$1.85 million

0.1% of SAM discharges
~ 370,000 discharges at $50/discharge

SOM Year 3

~$18.5 million

1% of SAM discharges
~ 370,000 discharges at $50/discharge

13x ROI Base

Proven ROI: Target HRRP hospitals facing CMS fines

Measuring success: readmission rate, discharge time saved per patient, CMS fine avoidance ($)



Thank You

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Desirability

For the Patient

Reduces Costs
- Patients don't have to pay for readmission

Prescription and Device Clarity
- Patients don't have to worry about confusing devices or roundabout prescriptions

Saves Booking Times
- Patients will have appointments automatically booked by default

For the Hospital

Reduces Readmission Costs
- Direct financial consequences for readmissions

Saves staff time
- Discharge coordination is highly manual → nurses and case lose time for patient care

Audit trail and compliance built in
- Every agent action is logged, PHI is encrypted, and BAAs are in place with all vendors

Integrates with existing workflows
- A layer on top of what hospitals already use

Fairness, Accountability, Transparency, Ethics

Human Oversight

- Every agent action is logged with a full audit trail
- No clinical action is confirmed without human sign-off
- Explainable recommendations — clinicians see what was flagged and why

Built For Trust

- Consistent protocols applied regardless of patient demographics
- Protected Health Information (PHI) encrypted, role-based access, Business Associate Agreements (BAAs) with all vendors
- Patients receive plain-language discharge summaries

Risks & Mitigations

	Risk	Mitigation
	HIPAA & Unauthorized Access — sensitive PHI collected at scale	BAAs with all vendors, encrypted pipelines, role-based access
	AI Inference Errors — wrong medication or treatment compiled	Human review required before any clinical action is confirmed
	EHR Integration — Epic approval is slow and hospital-by-hospital	Start with read-only Fast Healthcare Interoperability Resources (FHIR) APIs, write-back in later phases
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